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BSH GOOD PRACTICE PAPER

Point-of-care testing in general haematology: An update to the British Society for Haematology good practice paper

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KEYWORDS
blood gas, coagulation, D-dimer, deep vein thrombosis, governance, haematology, haematology testing, haemoglobinopathy test, International Organization for Standardization, limitations, point-of-care, reliability, sensitivity, specificity, Wells score

METHODOLOGY

This guideline update was compiled according to the British Society of Haematology (BSH) process at https://b-s-h.org.uk/media/or2db2zv/2024-08-20-bsh-guidelines-development-process.pdf?cf=638372962768270000. Levels of evidence and the strength of recommendations were evaluated using Grading of Recommendations Assessment, Development and Evaluation (GRADE) nomenclature (http://www.gradeworkinggroup.org).

Literature review details

Literature searches were conducted in January 2025 using PubMed. Only publications in English that included abstracts, with respect to studies conducted in humans, were included. Asterisks (*) in keywords captured tense and plural variants.

Searches were run, with a date range between 1 August 2019 and 16 January 2025 (to capture literature published after the previous guideline), using the following keywords: Point of care, haematology, haematology testing, sensitivity, specificity, reliability, International Organization for Standardization, Wells score, D-dimer, deep vein thrombosis, coagulation, blood gas, limitations, governance, haemoglobinopathy test*.

Further searches were run, with a date range between 1 January 2009 and 16 January 2025. These used the following keywords: fibrinogen test*, ROTEM, TEG, external quality assessment, proficiency test*. This update provides a more exhaustive description of fibrinogen testing, viscoelastic testing and quality than the previous update. Therefore, a broader date range was selected.

The following terms were excluded from all searches: viral load, human chorionic gonadotropin, beta-HCG, glycated haemoglobin (HbA1c), sickle cell, tuberculosis, malaria, human immunodeficiency virus (HIV), murine, case report, case-based update.

In total, 683 articles were identified. Additional articles deemed relevant were also reviewed.

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Review of the manuscript

Review of the manuscript was performed by the BSH Guidelines Committee—General Haematology Task Force, the BSH Guidelines Committee and members of the BSH sounding board.

INTRODUCTION

This good practice paper updates the BSH Guideline on point-of-care testing (POCT) published in 2019[1] and reflects the evolution of POCT.

POCT describes diagnostic testing outside the hospital laboratory, in close proximity to the patient, where results are available rapidly enough to influence immediate clinical management.[2,3] Recently, POCT delivery has been shaped by the confluence of innovation, strengthened frameworks, digital integration and connectivity.

Ongoing technological development has given rise to portable, user-friendly POCT platforms and assays with improved rapidity, reliability and analytical performance. POCT in diagnosis and monitoring can directly impact on clinical decision-making, translating to faster patient turnaround times (TATs). Moreover, POCT has become widely embedded across various healthcare settings, including acute hospital care, primary care and community pharmacies.

The International Organization for Standardization (ISO) standard, ISO 15189:2022, specifies quality, governance and risk assessment in medical laboratories.[4] Importantly, ISO 15189:2022 incorporates POCT requirements previously covered under ISO 22870:2016 (which is now superseded, although a few organisations may still hold accreditation to ISO 22870:2016).[3,4]

The coronavirus disease 2019 (COVID-19) pandemic accelerated decentralisation of testing and reinforced the value of rapid, accurate, near-patient diagnostics. Increasingly, devices are connected to laboratory information systems and electronic health records (EHRs), enabling data transfer and remote monitoring.

This good practice paper provides an overview of current POCT assays and sets out a framework for high-quality POCT in secondary care. POCT in community settings is outside the scope of this paper and will be addressed separately.

CLINICAL UTILITY OF POCT

Hospitals must balance safe, timely patient turnaround with quality of care. Rapid, accurate results afforded by POCT can facilitate patient flow and inform immediate treatment administration in life-threatening clinical settings.[1,5] POCT may be implemented within a wider strategy for operational efficiency, alongside effective management plans and waiting time targets, with the approval of a POCT committee.

RANGE OF EQUIPMENT AND ASSAYS

Table 1 summarises commonly available POCT assays and platforms in haematology and is not intended to be exhaustive. Description of assays does not constitute endorsement or promotion of any particular assay.

Full blood count and automated differential

Portable spectrophotometric full blood count (FBC) analysers that measure haemoglobin concentration (Hb) and haematocrit (Hct), such as i-STAT™ (Abbott) and DiaSpect™, provide vital information in various clinical settings.[6–8] Although the Hemocue® cannot measure Hct directly, this can be estimated from Hb measurement. Benchtop analysers combining flow cytometry with impedance counting (three- or five-part differential) can capture broader parameters beyond Hb/Hct, including platelet count (PLT), and total and differential white blood cell count.[11,12,30–32] More advanced analysers deploy artificial intelligence (AI)-enabled digital imaging to count and analyse cells.[33] Ongoing development of machine learning capabilities will likely expand the utility of analysers across clinical settings and facilitate more rapid, personalised patient care.

Several trials using venous and capillary blood samples have demonstrated accurate, reproducible results of FBC POCT analysers with comparisons made to central laboratory (CL) instruments.[6,11,34,35]

Point-of-care PT/INR testing for anticoagulation management

Prothrombin time (PT) and international normalised ratio (INR), essential parameters in anticoagulant monitoring, vary depending on whether patients are receiving direct oral coagulants (DOACs) or vitamin K antagonists (VKAs). Although DOACs are broadly used, warfarin or other VKAs are administered in limited indications with high target INRs, such as in patients with metallic heart valves.

Laboratory INR testing is often inconvenient, resource-intensive and inaccessible, limitations that were apparent during the COVID-19 pandemic.[36] The need for decentralised, patient-friendly PT/INR testing has given rise to various electrochemical and electromechanical POCT devices.[13,16] These include the CoaguChek® Pro II and XS (Roche), Xpercia Prime™, i-STAT, Micro INR and LumiraDX. Recently, smartphone-based technologies have been developed.[14,15]

Despite widespread use, portable PT/INR devices have notable limitations. Clinically significant discrepancies have been observed in patients with antiphospholipid syndrome receiving warfarin.[13] Moreover, at INRs ≥4, there is the need to monitor analytical bias between different test strips.[16] Within a stable therapeutic window (INR 2–4), POCT devices are comparable to laboratory-measured INRs ≤4.[13]

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TABLE 1 Commonly available POCT technologies and their clinical utility/settings.

Assay	Principle	Parameters	Clinical utility	Advantages and disadvantages
Spectrophotometric determination of Hb[6–8]	These assays involve determination of Hb in venous or capillary blood samples via photometric (spectrophotometric) measurement of light absorbance at specific wavelengths	Measuring Hb (e.g. i-STAT™, HemoCue® Hb 201+, EKF DiaSpect™)	Measurement of Hb in virtual wards, operating theatres, clinics, GP surgeries and blood donation centres	Portable Hb analysers provide quick results (1–3 min) with high reproducibility, but are susceptible to interferences by high levels of lipids (lipaemia) or bilirubin (icterus)
Impedance counting for FBC[6,9,10]	These assays operate according to the Coulter principle, whereby changes in electrical resistance are measured as blood cells are passed one-by-one through a small aperture	FBC, including RBC, WBC, PLT counts (e.g. Sysmex pocH-100i, Pixcell Hemoscreen)	Measurement of FBC parameters in virtual wards, emergency departments, operating theatres, GP surgeries, community clinics and clozapine clinics	Impedance counting is a well-established method that deploys simple hardware, although it is limited in the differential count (often three part or five part)
Impedance counting with five-part differential count and microfluidics[11,12]	These are novel analysers that combine five-part impedance counting of blood cells through microfluidic channels with high-resolution imaging and AI classification of cells	FBC (e.g. PixCell Hemoscreen,[11] Sysmex XNL[12])	In addition to FBC measurement, these advanced analysers are capable of detecting blood cell abnormalities. They are recommended for use in haematology clinics	Five-part differential analysers provide high resolution with minimal user input. Moreover, they can be bidirectionally interfaced, allowing for technical validation in the CL and real-time results reporting into patient records. However, they have a higher cost per test and are limited by cartridge supply
PT/INR Testing[13–16]	Clotting is activated with thromboplastin. POCT PT/INR analysers deploy an electrochemical or electromechanical detection principle, with sensors detecting ion movement related to clot formation	PT/INR for managing patients on anticoagulant therapy (e.g. CoaguChek XS Plus, i-STAT PT/INR cartridge)	Assessing the degree of inhibition of the vitamin K-dependent clotting factors in patients receiving VKAs	PT/INR testing is quick and easy to perform, utilising a small sample volume. However, it is unsuitable for patients with lupus anticoagulant or very high INR
Multiple electrode platelet aggregometry (impedance aggregometry) for platelet function testing[17,18]	Multiple electrode platelet aggregometry involves adhesion aggregation of platelets within a whole blood sample, leading to increased electrical impedance between the electrodes of the device	Platelet aggregation, including response to ADP and arachidonic acid (e.g. Roche Multiplate, VerifyNow)	Monitoring the efficacy of anti-platelet therapy in prevention and management of arterial thrombosis.	Although being a whole blood method simplifies operation, skilled interpretation of results is required. Reagents can be costly
Measurement of activated clotting time[19]	These devices measure clotting time following activation of the clotting cascade via a clotting activator such as kaolin	Measurement of ACT and aPTT (i-STAT ACTk cartridge, Hemochron)	Monitoring heparin dose adjustments in cardiac surgery and ECMO settings	Various factors can dissociate ACT from the anticoagulant effect of unfractionated heparin, including hypothermia, haemodilution and platelet function
Measurement of primary haemostasis via platelet aggregation[20,21]	Whole blood is drawn through a 200-μm diameter capillary by applying negative pressure. At one end of the capillary is a membrane coated with collagen and adenosine diphosphate (PFA-ADP) or epinephrine (PFA-Epi), which interacts with platelets. Occlusion of the capillary aperture through aggregation is indicative of primary haemostasis[21]	The closure time (CT) of the aperture through adhesion and aggregation of the platelets is measured and is indicative of primary haemostasis (PFA-100/200)[21]	Monitoring platelet dysfunction during surgery	Non-selective screening with the PFA has no predictive value for bleeding or transfusion requirement in patients undergoing invasive procedures
TEG/ROTEM or Quantra for viscoelastic detection[22,23]	TEG/ROTEM measures changes in clot strength as a pin rotates in a blood sample within a cup (mechanical resistance). Quantra uses ultrasound to measure changes in clot stiffness	Real-time coagulation dynamics are assessed (e.g. TEG 5000, TEG 6S, ROTEM Delta, ROTEM Sigma, Quantra QPlus). The impact of functional fibrinogen may be assessed with CFF (with TEG) and FIBTEM (with ROTEM)	TEG/ROTEM or Quantra are used to guide transfusion decisions, providing information on fibrinogen, platelets and clot lysis. Viscoelastic testing can be used to assess coagulation dynamics in clinical settings relating to trauma-induced coagulopathies, cardiac surgery, obstetrics, cardiothoracic surgery, liver disease, emergency department and extra-corporeal membrane oxygenation settings	TEG/ROTEM and Quantra can provide real-time feedback (10–15 min TAT). However, they are limited by the need for costly equipment with complex set-up and maintenance

(Continues)

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TABLE 1 (Continued)

Assay	Principle	Parameters	Clinical utility	Advantages and disadvantages
The ClotPro for viscoelastic detection[24]	The set-up is similar to ROTEM/TEG, wherein the blood sample is placed in a cup with a pin, except the cup rotates while the pin is stationary. Changes in viscoelastic properties indicate clot formation.[24] This is done in conjunction with the ECA test or RVV test. Haemostasis is assessed using real-time coagulation dynamics via the ECA test or RVV test	ClotPro is used for assessment of haemostasis using real-time coagulation dynamics. ClotPro can detect the presence and concentration of DOACs	The ECA test (for assessing the activity of direct thrombin inhibitors) can be used to monitor dabigatran and argatroban. The RVV Test (for assessing activity of factor Xa inhibitors) can be used for detection of enoxaparin, rivaroxaban and apixaban	Differentiation between anticoagulant classes is not always clear
Turbidimetric or latex agglutination immunoassays[25,26]	These immunoassays work on the principle that antigen–antibody complex formation causes light scattering or turbidity	During clotting, the soluble protein fibrinogen is converted into insoluble fibrin. D-dimers, heterogeneous fragments generated by breakdown of cross-linked fibrin, provide an indirect marker of coagulation. Various immunoassays can be used for assaying D-dimer or fibrinogen (e.g. Radiometer AQT90, i-Stat D-dimer, Lumira DX, Exdia TRF, Cobas H232, Triage)	Immunoassays can be used for assessment of DVT or PE	Expands the scope of coagulation testing at the bedside but is limited in decentralised environments
Lateral flow Immunoassay for detection of haemoglobin variants[27]	In the HemoTypeSC, monoclonal antibodies specific to haemoglobin variants HbA, HbS and HbC are immobilised in distinct lines on a nitrocellulose strip	Detection of HbS, HbC and HbA (e.g. HemoType SC)	HemoTypeSC is used to detect the presence of the most common forms of sickle cell disease (HbSS and HbSC)	HemoTypeSC can detect HbS, HbC and HbA but not other clinically significant haemoglobin variants
Microchip electrophoresis for beta-thalassaemia screening[28]	Gazelle is a miniaturised haemoglobin electrophoresis assay with a single-use cartridge containing a cellulose acetate test strip. Different haemoglobin fractions migrate and separate into visible bands on cellulose acetate strip for quantitation.[28] Cation-exchange high-performance liquid chromatography identifies haemoglobin variants using their retention time after separation by cation exchange chromatography[28]	Separation of haemoglobin fractions via electrophoresis or cation exchange enables identification (e.g. Hemex Health Gazelle)[28]	The Gazelle electrophoresis assay is used for screening for beta-thalassaemia trait	Although high sensitivity, specificity and accuracy have been observed,[28] post-screening confirmatory testing is required. Cellulose acetate electrophoresis is not recommended as a reliable method of quantitation
Enzymatic G6PD deficiency screening[29]	This is a quantitative enzymatic assay, in which NADPH production from G6PD activity is measured via change in optical absorbance at 340 nm. Normalisation of measured NADPH to Hb is subsequently used to differentiate G6PD deficiency from normal status	Assaying G6PD (e.g. STANDARD™ G6PD test)	Deployed for G6PD deficiency screening	Measurement of G6PD activity may be confounded by factors such as gender or reticulocyte count. Variability in G6PD deficiency values between different laboratories may lead to varying results. This emphasises the need for harmonisation, consistent with ISO requirements

Abbreviations: ACT, activated clotting time; ACTk, activated clotting time-kaolin cartridge; ADP, adenosine diphosphate; AI, artificial intelligence; aPTT, activated partial thromboplastin time; CL, central laboratory; CFF, citrated functional fibrinogen; CT, closure time; DOACs, direct oral anticoagulants; DVT, deep vein thrombosis; ECA, Ecarin chromogenic assay; ECMO, extra-corporeal membrane oxygenation; FBC, full blood count; FIBTEM, fibrin-based thromboelastometry; G6PD, glucose-6-phosphate dehydrogenase; GP, general practitioner; Hb, haemoglobin concentration; INR, international normalised ratio; ISO, International Organization for Standardization; NADPH, nicotinamide adenine dinucleotide phosphate; PE, pulmonary embolism; PFA, Platelet Function Analyser; PLT, platelet count; POCT, point-of-care testing; PT, prothrombin time; RBC, red blood cell; ROTEM, rotational thromboelastometry; RVV, Russell's viper venom; SC, sickle cell; TAT, turnaround times; TEG, thromboelastography; VKA, vitamin K antagonist; WBC, white blood cell.

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Point-of-care activated clotting time testing

Activated clotting time (ACT) is used to monitor unfractionated heparin therapy in cardiac surgery and extra-corporeal membrane oxygenation (ECMO) settings.[37,38] The i-STAT ACTk (ACT-kaolin) cartridge and the electromechanical Hemochron™ (Werfen) are POCT devices capable of measuring ACT.[19]

Reportedly, ACT correlates strongly with anti-Xa activity in non-cardiac arterial procedures, as well as cardiac surgery with incomplete protamine-induced heparin reversal.[37,38] However, various factors can dissociate ACT from the anticoagulant effect of unfractionated heparin, including hypothermia, haemodilution and platelet count and function.[37] In the cardiopulmonary bypass setting, ACT correlates poorly with anti-Xa activity in patients with lupus anticoagulant (prolonged activated partial thromboplastin time [aPTT]) or elevated heparin-von Willebrand factor.[39,40] In a comparison study, the i-STAT demonstrated better reproducibility of results, while the Hemochron showed higher variability and lower concordance.[19] Given the variability in ACT results, it is imperative that device-specific thresholds are established.

Point-of-care D-dimer testing

D-dimer testing is deployed in the diagnostic algorithm to rule out deep vein thrombosis (DVT) and pulmonary embolism (PE), alongside a clinical probability index score (e.g. Wells score, Geneva score, Pulmonary Embolism Rule-out Criteria [PERC]) and age-adjusted reference ranges. Laboratory D-dimer testing is based on latex immunoassays, or less commonly, the enzyme-linked immunosorbent assay (ELISA)-based VIDAS system. However, POCT immunoassay devices enabling (qualitative or quantitative) D-dimer testing have been introduced. These include Cobas (Roche®), Triage (QuidelOrtho™), AFIAS-1®, Standard F200®, LumiraDx™, HP-AFS/1® (Hipro) and the recent Exdia TRF Plus (Precision Biosensor Inc).[25,26]

The Triage test has demonstrated diagnostic accuracy comparable to the VIDAS test.[25] A clinical evaluation study demonstrated low false-negative rates (<1.4%) for Exdia TRF Plus, AFIAS-1®, Standard F200®, LumiraDx™ and HP-AFS/1® platforms, with comparable performance to a laboratory assay (STA® Liatest®).[26] However, a meta-analysis showed POCT D-dimer assays to have slightly lower sensitivity than laboratory assays, emphasising the need for their use only in low-risk populations.[41] The findings indicate that qualitative POCT D-dimer assays have limited utility, due to low sensitivity.[41] Moreover, the performance of qualitative assays may be further limited by quality issues.

Viscoelastic methods for assessment of global haemostasis

Viscoelastic assays are indispensable for managing acute bleeding and haemostatic challenges in trauma, cardiac surgery and obstetrics settings.[22,23,42,43] POCT devices include thromboelastography (TEG®; Haemonetics), rotational thromboelastometry (ROTEM; Werfen), Quantra® (Stago), Sonoclot® (Sienco), ClotChip® (XaTek® Inc.) and ClotPro (enicor GmbH, Munich, Germany; Haemonetics Corporation).[22,23,42,43] Operational designs incorporate 'cup and pin', ultrasonic detection and cartridge-based set-ups.[23]

Viscoelastic assays generate data more rapidly than alternatives, translating to turnaround efficiencies. Quantra, TEG 6S and ROTEM Sigma devices have shown comparable results (strong correlations) with routine laboratory testing for platelet and fibrinogen levels.[22] Diagnostic performance was reportedly similar between Quantra and ROTEM in the trauma-induced coagulopathy setting, and superior for ROTEM in the hypofibrinogenaemia setting, although Quantra generated faster results.[22] Testing algorithms incorporating Quantra, TEG or ROTEM are shown to enhance the quality of transfusion management, leading to modest outcome improvements in patients experiencing bleeding.[23,44]

The National Institute for Health and Care Excellence (NICE) recommends use of TEG and ROTEM in cardiac surgery, whereas Sonoclot is only recommended in research.[45] Moreover, ISO 15189:2022 (Section 7.3.5) recommends establishment of viscoelastic device-specific algorithms for clinical decision-making and reference ranges for local clinical practice.[4]

Point-of-care direct oral anticoagulant testing

Direct oral anticoagulants such as inhibitors of factor IIa (e.g. dabigatran) and factor Xa (e.g. apixaban, edoxaban, rivaroxaban) are used to prevent and treat thromboembolic events. Although monitoring DOAC levels is not considered necessary, measuring patients' DOAC levels is advantageous for clinical decision-making in stroke, bleeding, urgent surgery and pre-thrombolysis settings.

Direct thrombin monitoring (DTM), DOASENSE™ (DOASENSE GmBH), CoaguChek® and TEG 6S assays cannot quantify DOACs, but may rule out clinically relevant levels (<30–50 ng/mL). The DTM test card showed a high correlation between dabigatran levels and ecarin clotting time (ECT) in whole or citrated blood, with 95%–97% sensitivity for >30 and >50 ng/mL clinical cut-offs.[46] DOASENSE is a chromophore-containing colorimetric urine dipstick. Reported sensitivities of DOASENSE for dabigatran and factor Xa inhibitors were 100% and 97%,

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respectively, at levels ≥30 ng/mL.[47] It was reported that CoaguChek INR has >95% specificity for edoxaban levels >30 and >50 ng/mL, which could guide thrombolysis or reversal therapy in post-ischaemic stroke recovery.[48] Testing algorithms that incorporate TEG 6S may aid decision-making in trauma and acute care surgery.[49] A DOAC identification algorithm for TEG 6S demonstrated 98%–100% sensitivity for Xa and IIa inhibitors and 100% specificity in a study of patients with or without DOACs and healthy donors.[49]

Blood gas Hb/haematocrit measurement

Modern blood gas analysers (BGAs) can measure Hb (spectrophotometrically) and Hct (via conductivity or calculation). As BGAs analyse whole blood, haemolysis cannot usually be detected and should be considered when interpreting unexpected or discordant results.[50] Various studies show agreement between Hb measurement by BGA and CL analysers.[51,52] However, underestimation of BGA-determined Hb/Hct compared with CL measurement may lead to unnecessary trigger activation of blood transfusion.[53–56] One study showed Hb/Hct underestimation with the centrifugal BGA reference method recommended by the Clinical & Laboratory Standards Institute (CLSI).[54] Another study concluded that 21% of patients who would have received transfusion based on CL-determined Hct would otherwise not be transfused based on BGA-determined Hct.[56] Conversely, systematic overestimation of Hb/Hct with BGAs has been observed (mean 8 g/L vs. CL).[57] These findings suggest that BGAs are not interchangeable with CL analysers for Hb measurement, based on Clinical Laboratory Improvement Amendments (CLIA) criteria for interchangeability.[57,58]

Point-of-care testing for assessment of anti-platelet therapy

Point-of-care platelet function assays are available, based on various operating principles. These include turbidometric optical detection, multielectrode aggregometry, clot closure and automated cell counting. Devices include platelet function analyser (PFA-100/200), VerifyNow™ (Werfen), Multiplate (Roche), PL-11, PlateletWorks® and Smart Clot (16.12 Medical Technologies).[17,18] Additionally, TEG can be adapted for platelet testing.[17,18]

The performance and variability of platelet tests are important considerations. Notably, the PFA screening test for platelet function is non-specific and cannot differentiate between von Willebrand disease and platelet function abnormalities.[17] A study comparing the PL-11 platelet count drop method against light transmission aggregometry, the VerifyNow™ aspirin system and TEG, in samples from healthy subjects, showed marked inter-device variability.[18]

Studies have shown that prediction of perioperative risk of bleeding and transfusion is accurate with multiplate aggregometry (in cardiac surgery)[17,59–61] but poor with PFA screening (in cardiac and other invasive surgeries).[20,21,62–64] In coronary artery bypass grafting, multiplate aggregometry outperformed TEG as predictors of bleeding, transfusion requirements and post-surgical re-exploration.[17] A meta-analysis investigating point-of-care platelet testing in cardiac surgery, involving 39 studies ($n = 4101$), further supports the predictive value of multiplate aggregometry.[65] The meta-analysis supports the general utility of point-of-care platelet tests in predicting blood loss and transfusion requirements but notes interdevice variability.[65] This may complicate the setting of recommended clinical decision limits for tests.

Point-of-care fibrinogen testing

Various POCT devices are capable of quantifying fibrinogen. The qLabs® FIB system (Stago), FibCare (Atom Medical) and the F-Point (X-Point Diagnostics) have shown comparable results to the Claus method.[66–68] The operational principle of the FIB system is based on electrical impedance, whereas Fibcare and F-point devices measure light scattering of particles.[66–68] The operational advantages of these assays, including speed, versatility and reduced laboratory testing, are particularly useful in transfusion, haemorrhage and gynaecological surgery settings.[66–68]

TEG and ROTEM are reportedly effective for detecting hypofibrinogenaemia, via citrated functional fibrinogen (CFF) and fibrin-based thromboelastometry (FIBTEM) parameters.[69,70] These findings suggest that viscoelastic testing may be suitable for guiding individualised treatment in postpartum and surgical bleeding.[69,70]

Recommendations

- **Device-specific thresholds should be established for ACT tests, given the variability between tests (2B).**
- **Only high-sensitivity D-dimer tests with performance matching STA® or ELISA laboratory assays should be used (1B).**
- **Validation of viscoelastic assays should involve establishment of clinical decision-making algorithms and reference ranges (1B).**
- **Blood gas analysers should not substitute laboratory-based testing of Hb/Hct but may aid in monitoring trends or making immediate clinical judgements (1C).**
- **Where interfacing of results from POCT to a laboratory information management system (LIMS) exists, this should be utilised in real time to prevent transcription errors (1C).**

QUALITY ASSURANCE IN POCT

Key to the delivery of an accurate, reliable POCT programme is robust quality assurance (QA) which covers pre-analytical,

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analytical and post-analytical aspects of POCT. A QA programme should encompass training, personnel, equipment and appropriateness of testing.[71] The POCT service should be integrated into the CL quality management system, adhering to the same standards of professional leadership, training, competency, IT connectivity, procurement, governance and professional monitoring. Although not essential for accreditation, a quality manual can inform staff and users of the agreed service delivery structure.

Recommendations
The POCT QA framework should include (1B):

- **Oversight and monitoring by a laboratory professional or quality manager**.
- **Internal quality control (IQC) and external quality assessment (EQA) processes**.
- **An internal audit schedule**.
- **Accreditation to ISO 15189 by the appointed national accreditation bodies**.
- **Document control including standard operating procedures (SOPs)**.
- **Regular review of the service by senior management for continual improvement**.

Internal quality control programme

An IQC programme should control accuracy, variability (within and between-day), drift and bias. This includes controls that parallel clinically significant and decision-making thresholds, defined by national and local clinical policies.[72]

The frequency of IQC analysis should be based on patient analysis activity. Drift in IQC should be analysed at intervals defined by local policy.[4,73] Records of lot numbers, target values and IQC results should be fully traceable (whether electronic or written).

Where possible, IQC and calibration checks should include third-party QC material, allowing independent device performance verification. IQC should be run following instrument maintenance or calibration (where possible) and to verify new reagents or consumables (batch acceptance). There should be a policy to handle IQC failure and a contingency plan for system failures.[4]

External quality assessment

External quality assessment (EQA) provides samples of a known but undisclosed analyte concentration for assessment, reflective of clinical decision-making levels and the device linearity. The frequency of EQA varies according to the device and programme design. This should complement but not replace IQC.

The EQA report provides users with peer-reviewed assessment via numerical or qualitative scoring of device performance, relative to other users of the same device or method. This provides a long-term retrospective review of system performance. Results comparable between EQA material and patient samples across different platforms can inform post-market surveillance.[74]

There should be a system for documenting EQA results and for responses to out-of-consensus performance. A difference between the result and the target value, while statistically out-of-consensus, may not be clinically significant. The user should interpret this in the context of overall, long-term analytical performance and the clinical significance of concentration discrepancies.

EQA is not available for all devices or analytes. Alternatives can include interlaboratory exchange of blinded samples within a health trust, and simultaneous collection of venous and capillary samples for multiple device analysis.[75] Limits of agreement, set according to local clinical and analytical thresholds, should reflect physiological differences (e.g. venous vs. capillary samples). Periodic testing of POCT devices against CL methods is good practice.

Users of POCT devices are not laboratory scientists.[76] Analysis of EQA and IQC analytes should be encouraged among regular POCT users, not just experienced staff or laboratory staff. Analytes for EQA often differ from clinical samples (e.g. whole blood vs. capillary blood), which may lead to errors stemming from users' unfamiliarity with EQA. Appropriate training can mitigate against such errors.

Accreditation and audits

Internal audits are instrumental to monitoring compliance and improving quality. The audit schedule should cover all relevant aspects of the service.[4]

Audit findings alongside corrective and preventative actions should be incorporated into the POCT policy and procedures. In the United Kingdom, hospital POCT is accredited as part of the clinical laboratory by the UK Accreditation Service (UKAS).[77]

Standard operating procedures

Standard protocols should be developed and made available to all POCT service staff. The structure, content and format of POCT SOPs should meet requirements outlined in ISO 15189.[4] Elements of a POCT service detailed in SOPs are shown in Figure 1. Results should be filed permanently (in electronic or paper patient records), clearly identified as a POCT result.

MANAGEMENT AND GOVERNANCE

POCT devices must not be adopted unless there is a well-defined governance structure, alongside clear accountability for all service elements.

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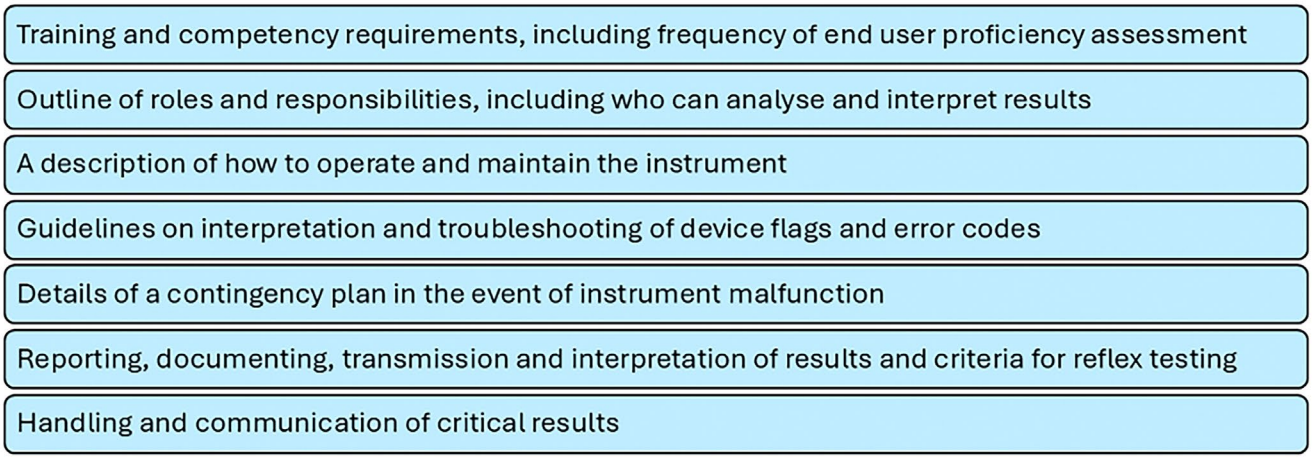


FIGURE 1 Elements of a POCT service to be covered in the content of SOPs, in accordance with requirements laid out in ISO 15189.[4] POCT, point-of-care testing; SOP, standard operating procedure.

Stand-alone POCT services are rare unless provided commercially; most represent a subsection of an accredited pathology laboratory under clinical directorship. Day-to-day operation may be delegated to a POCT coordinator or manager. Governance may be provided by a multidisciplinary POCT committee or governance group, although not mandatory according to ISO 15189:2022.[4] Membership must be carefully selected, with appropriate terms of reference.[4] Members should include key healthcare staff, including clinicians, laboratory scientific staff, nurses, healthcare support staff and management, the POCT coordinator or manager and representatives from Information Technology (IT), Estates and Clinical Engineering departments. Members are expected to demonstrate expertise in quality management and medical devices, gained through training. They must also seek advice on finance and procurement, when appropriate. Their accountability should be to a higher level governance group within the organisation, to which issues can be escalated.[4] If POCT extends to the community, the committee should include professional community representation (e.g. a pharmacist, general practitioner or practice nurse) able to convey the community's needs to the committee, integrated care boards or integrated care system.

A POCT committee or governance group may not be feasible. Nevertheless, clinical and operational effectiveness must still be demonstrated via the POCT policy and evidence generated.

Where the POCT service coverage is provided outside an accredited organisation, a Service Level Agreement (SLA) must be in place, detailing responsibilities and expectations of all parties involved in service delivery.[2,3,78]

Recommendations

- **The POCT policy must describe governance, quality assurance and risk management arrangements for the implementation of a new test or device (1A).**
- **Where devices or tests may not meet traditional quality and governance requirements (e.g. emerging technologies or methodologies, absence of a laboratory comparator), formal risk assessment must be undertaken. Identified risk must be mitigated where reasonably practicable (1B).**

TRAINING AND DEVELOPMENT

The elements that should be covered in a POCT training programme are shown in Figure 2.

Mandatory retraining intervals must be stated in the POCT policy. The stipulations of the policy are enforceable on all users. It is imperative that the policy is approved ('signed off') at a high level within the organisation's governance structure.

The training and development programme should be auditable, including actions addressing poor user performance and reducing risks to patients. Audits must be conducted at least annually, although higher risk tests should be reviewed more frequently, especially if there are trends in poor performance. Continuous improvement and preventive actions should be based on user feedback, audit findings, trend analysis and clinical incident reporting. This should inform training updates, version control and process changes.

Recommendations

- **A structured training programme (theoretical and practical) for all POCT personnel and end users must be developed and maintained by the POCT manager or training and development coordinator (1A).**
- **All training materials (structured syllabi) must be alongside a competency assessment, to be completed and passed before first use. A training manual may ensure a structured approach (1A).**
- **Before training or assessing end users, POCT personnel must first demonstrate a higher level of understanding of the test process, application and performance (1B).**

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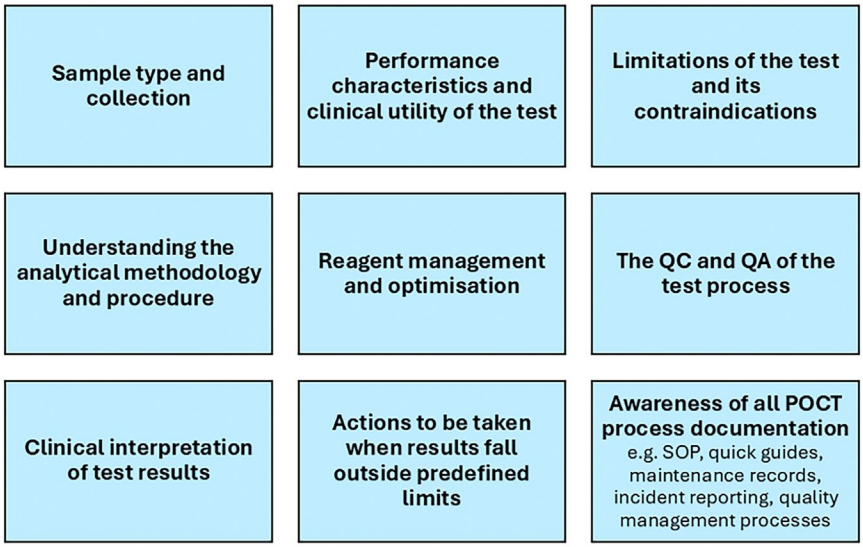


FIGURE 2 Elements that should be covered in a POCT training programme, applicable to all assays and devices. POCT, point-of-care testing; QA, quality assurance; QC, quality control; SOP, standard operating procedure.

- **A separate training and competency document must be maintained and updated whenever a point-of-care test or device changes (1A).**
- **Training complexity and competency is proportional to the clinical application of the test or process (1B).**
- **Training and competency assessment should cover expertise required to safely provide results for patient care (1A).**
- **Users must demonstrate competence in device operation, methodological theory, pre-analytical, analytical and post-analytical factors, and result validity (1A).**

FINANCE AND COST–BENEFIT ANALYSIS

Historically, POCT was perceived to be more expensive than equivalent laboratory tests. However, various analyses have suggested an overall financial benefit of introducing POCT.[79,80] As well as direct cost savings, avoidance of additional costs by introducing a test should be considered (e.g. avoidance of cancellations, unnecessary appointments, readmission, specialist appointments or additional appointments, reduced length of stay [LOS], optimised imaging and theatre time). A major driver of cost savings from POCT is staff time optimisation.[80]

Service providers must understand the true cost of POCT across the service. Costs attributable to POCT, shown in Figure 3, are easily quantifiable through quotes, supplier agreements or fixed pricing contracts.

POCT services must work closely with procurement and performance teams to determine the true cost–benefit ratio. Accurate cost analysis requires knowledge of costs of LOS or admission to a hospital bed in any given specialty. Service leads must have strong business acumen and experiential knowledge of their organisation's financial systems. Services must demonstrate financial control, identifying new financial risks early and providing tangible solutions.

Service coordinators and managers must be able to produce compelling business cases for executive approval and funding bids. A standard template can be used which quantifies the existing risk to patient care, and how a POCT solution will positively impact that care. This should also outline the organisation's required financial commitment, and how impacts on care demonstrate cost-effectiveness.

Service leads may consider POCT provision within a managed service partnership with the commercial sector. This can lead to financial and operational benefits, most notably mitigation of service provision risks. However, financial efficiencies should be evaluated against patient outcomes to ensure the best service.

Recommendations

- **A financial framework should detail the cost-effectiveness and financial value in POCT provision as an alternative or complementary to the medical laboratory (1B).**
- **Recruitment should cover the necessary business skills to manage and develop the service as a business unit, or these skills should be developed internally (1B).**

EQUIPMENT SELECTION AND VERIFICATION

Selection of POCT devices requires consideration of various factors. This involves comprehensive review of product literature and specifications, including on-site demonstration and evaluation. Importantly, sample collection

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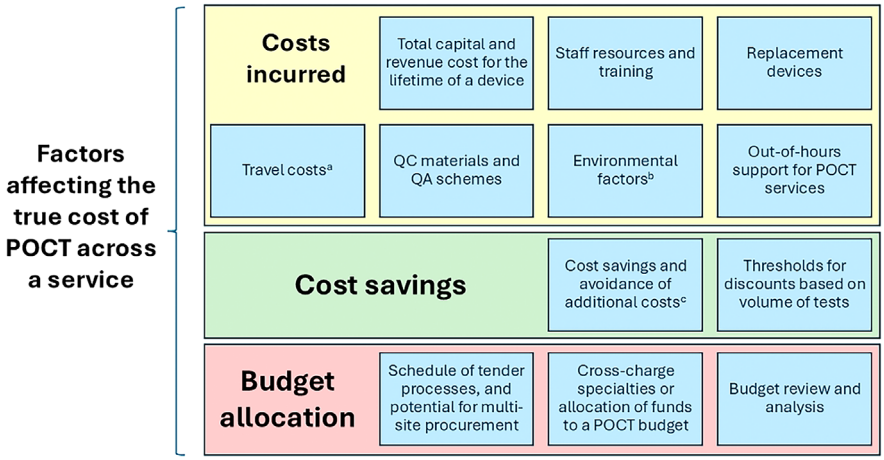


FIGURE 3 Factors affecting the true cost of introducing a POCT test or device across a service. [a]Travel costs should be considered if POCT services are away from a centralised location. [b]POCT assays are amenable to bulk ordering and reduced costs attributed to freight and wastage. Less frequent changes to lot numbers of components (e.g. reagents, cartridge and test strips) can lead to reduced use of product and resources for verification. [c]Introducing POCT may result in cost savings from optimised use of time and resources (e.g. optimised imaging slots, surgical efficiencies, reduced LOS) and avoidance of costs that would otherwise be incurred (e.g. cancelled appointments, additional appointments). LOS, length of stay; POCT, point-of-care testing; QA, quality assurance; QC, quality control.

should minimise blood loss to reduce patient discomfort and risks, especially in neonates and anaemic patients.[81] Frequent blood withdrawal can result in iatrogenic anaemia necessitating transfusion, leading to transfusion-acquired illness, infections and complications.[82]

The selection and verification of all POCT devices should be guided by the criteria proposed by Menendez-Botet.[81]

Recommendations

- **Clinical and medical effectiveness: Devices must demonstrate clear clinical utility by supporting diagnosis, treatment and patient management, ideally within established care pathways (1B).**
- **Economic evaluation: Economic impacts of devices should be assessed before adoption, including cost-effectiveness (per diagnosis or outcome), LOS reduction and workforce improvements, including productivity and satisfaction (1B).**
- **Patient safety and blood conservation: Devices should minimise blood volume requirements (1B).**
- **Safety and ergonomics: Devices must incorporate biohazard containment, safe disposal (sharps and waste), user-friendly design and risk reduction mechanisms (1C).**
- **Technical quality and reliability: Devices should have proven technical robustness and be amenable to automated calibration, reliable throughput, reagent stability, minimal pre- and post-analytical steps (where possible), accreditation and user training (1B).**
- **Analytical performance verification: Each device must be verified for accuracy, level of imprecision compared to the 'gold standard' method, reproducibility, linearity and stability across the measurement range. Results must be comparable with CL standards and include error detection, interference warnings and specimen flagging capabilities (1B).**
- **Data management and systems integration: Devices must support secure, long-term, interoperable data management and storage, for example, bidirectional connectivity, remote access, automated critical result alerts, EHR integration (1B).**
- **Device qualification: An ISO 15189:2022-compliant acceptance process must include verification protocols, method comparisons and training and competency requirements (1B).[4]**

HEALTH AND SAFETY

Health and safety measures for POCT, listed in Table 2, are critical to protecting patients and users.

Up-to-date documentation ensures safety, quality and regulatory adherence, including SOPs for device use and cleaning, risk assessments, Control of Substances Hazardous to Health (COSHH) assessments, quality control (QC) logs and service records. All documentation must align with Medicines and Healthcare products Regulatory Agency (MHRA) guidance, ISO 15189:2022 standards and Care Quality Commission (CQC) requirements.[4,83–85]

Recommendations

- **Staff must have up-to-date training and competency assessments and participate in risk assessments and audits (1A).**
- **Needle-stick injuries must be reported, for example, via Datix reporting system (1A).**
- **There must be disposal procedures for sharps, blood-stained material, reagents and electronic waste (1A).**

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TABLE 2 Health and safety measures for POCT.

Health and safety measure	Specific actions
Biological risk management	PPE, safe sharps disposal, treating all blood samples as potentially infectious
Infection prevention and control	Strict hand hygiene, cleaning devices with approved disinfectants between patients, maintaining clean and dirty work zones, single-use consumables
Chemical and reagent safety	Up-to-date COSHH assessments, correct storage of hazardous substances, access to spill kits with trained operators
Electrical safety	Annual PAT testing, keeping equipment dry and clean, scheduled servicing and calibration and promptly removing damaged devices from use

Abbreviations: COSHH, Control of Substances Hazardous to Health; PAT, performance acceptance testing; PPE, personal protective equipment.

REFLEX TESTING

Reflex testing in POCT refers to automatic or protocol-driven ordering of a follow-up test based on an initial result, without requiring a separate clinical order. For example, if HemoCue-determined Hb <80 g/L, the reflex may be the automatic ordering of FBC and ferritin for CL processing. Staff should be trained to ensure correct application, escalation and override when needed.

Reflex testing can translate to clinical benefits. These include reduced diagnosis and treatment delays, protocol adherence, improved patient flow in emergency and ambulatory care, and preventing missed diagnoses (e.g. thrombocytopenia or sepsis). However, robust governance, leadership and SOP documentation are needed.

ADVERSE INCIDENT REPORTING

Adverse incidents refer to any event or error involving a POCT device, result or process that could compromise patient safety or quality of results. These can occur at any stage of the process. Examples of adverse incidents are shown in Table 3. Prompt, systematic reporting is essential for identifying trends, corrective actions and compliance with the MHRA yellow card scheme, ISO 15189:2022 and CQC standards.[4,83–86]

Errors in POCT are a recognised cause of avoidable, delayed or inappropriate transfusion. Reports to the Serious Hazards of Transfusion (SHOT) haemovigilance scheme have highlighted various such incidents.[87] These include poor practice with respect to sample preparation for BGAs (e.g. inadequate mixing, partial clotting), which may lead to inaccurate Hb measurements or misinterpretation of results. These errors can result in inappropriate transfusion decisions and patient harm, including transfusion-associated circulatory overload (TACO).[87] Careful attention to sampling, analysis and interpretation and the reporting of relevant adverse incidents to SHOT are essential to supporting wider learning and risk reduction.

Adverse incident reporting is the responsibility of anyone involved in POCT, including healthcare professionals, biomedical scientists, laboratory assistants and pharmacists. Incidents must not be ignored even if a patient is not harmed. Events that identify an ongoing risk to patient care must be recorded in the organisation's Risk Register with appropriate actions managed at regular intervals. This may lead to business cases for service improvement to reduce patient risk to an acceptable level.

TABLE 3 Examples of adverse incidents.

Adverse incident	Examples
Device malfunctions	Hardware or software failures leading to inaccurate results (false high or low values for Hb or INR)
Operator errors	Insufficient sample volume, expired reagents, failing to follow SOPs
QC failures	Testing continues despite out-of-range QC
Data or connectivity issues	Results fail to upload correctly to EHRs
Infection risks	Cross-contamination between patients
Incidents arising from delayed actions	Missed reflex testing or failure to escalate critical values
Sample misidentification	Tests performed on the wrong patient or without proper identity verification
Errors leading to avoidable, delayed or inappropriate transfusion[a]	Errors in sample preparation or poor practice with BGAs, yielding inaccurate or misinterpreted results, leading to inappropriate transfusion decisions

Abbreviations: BGA, blood gas analyser; EHR, electronic health record; Hb, haemoglobin concentration; INR, international normalised ratio; QC, quality control; SHOT, Serious Hazards of Transfusion; SOP, standard operating procedure.

[a]Such errors must be reported to the SHOT scheme.

The following recommendations should be implemented:

- **A no-blame reporting culture to encourage incident reporting without fear of reprisal, supporting transparency and safety improvement (1B).**
- **Regular training on incident reporting (focusing on 'how' and 'when'), for all POCT users, to support clarity and confidence in the reporting system (1B).**
- **Clear reporting responsibilities embedded in SOPs specifying who should report, what should be reported and how to escalate concerns (1B).**
- **Proactive reporting to inform service improvements and targeted retraining, to prevent recurrence (1B).**
- **Routine audits of incident data to identify trends, share organisation-wide learnings and strengthen governance (1B).**

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- **Incidents that impact on transfusion practice should be reported to the Serious Hazards of Transfusion (SHOT) haemovigilance scheme**.

DATA PROTECTION

Data protection in POCT must comply with General Data Protection Regulation (GDPR) and the NHS Data Security and Protection (DSP) Toolkit.[88,89] Patient data should remain confidential and accessible only to authorised personnel. Data integrity must be protected to prevent corruption or unauthorised alteration. Information available to those with legitimate access should be collected and retained only to the minimum extent necessary. Records must be stored securely, archived and eventually destroyed. Strong security measures (e.g. encryption, passwords, firewalls, audit trails) should protect against breaches and unauthorised access.

Recommendations
The following should be maintained:

- **Secure, traceable, auditable digital documentation using centralised POCT management software, to reduce the risk of data loss or unauthorised access (1B)**.[3]
- **A document control system to ensure that only current, approved versions of policies, logs and reports are in use, precluding outdated versions (1B)**.[4]
- **Training for all POCT users to cover data protection, confidentiality, GDPR requirements and NHS DSP Toolkit standards (1A)**.[88,89]
- **Data access logs, audited at defined intervals to monitor inappropriate access or breaches and demonstrate data governance compliance (1B)**.
- **Secure destruction of expired paper records (e.g. cross-cut shredding, approved confidential waste disposal) in line with the NHS Records Management Code of Practice (1B)**.[90]
- **Compliance with GDPR, NHS DSP Toolkit and local information security policies should be overseen by an information governance lead or POCT data controller (1B)**.[3,90]

DOCUMENT STORAGE AND ARCHIVING

Records must be stored in a secure, access-controlled system for a defined retention period. Upon expiry of the retention period, records must be disposed of, following NHS data destruction and GDPR principles.

Recommendations

- **Patient test results should be retained for at least 8 years after the last patient contact, in line with NHS record-keeping requirements (1B)**.
- **Calibration records, QC logs and equipment maintenance reports are kept for a locally documented period (usually at least 5 years) to support audits, accreditation and incident investigations (1B)**.[4,91]
- **Training and competency records should be retained for at least 5 years to evidence staff authorisation and compliance (1B)**.

Key recommendations

- **Device-specific analytical and clinical thresholds should be established where interdevice variability is demonstrated (1B)**.
- **POCT services must operate a QA programme that includes IQC and participation in EQA (1B)**.
- **SOPs must be made available to and followed by all POCT staff (1A)**.
- **POCT patient results must be retained permanently (electronic or paper records) and clearly identified as a POCT result in the patient record (1B)**.
- **A structured programme of training, competency assessment and re-certification must be implemented for all POCT users (1A)**.
- **A financial framework must be developed to demonstrate cost-effectiveness and financial value of POCT services (1B)**.
- **Adverse incidents must always be reported, including near misses and events not resulting in patient harm (1A)**.
- **Centralised POCT management software should be implemented to ensure secure, traceable, auditable digital documentation (1B)**.
- **POCT records must be stored securely and retained for defined, policy-compliant retention periods (1B)**.

SUMMARY

POCT continues to evolve, offering rapid diagnostic results that directly influence clinical decisions. The publication of ISO 15189:2022 marks an important development, with increased focus on quality, risk assessment, patient-centred care and governance.[4]

A high-quality POCT service relies on robust technology, leadership, well-defined clinical pathways, training and QA. These elements must be embedded within an accredited framework to ensure the same standards as laboratory-based testing. Compliance with ISO 15189:2022 provides a foundation for safe, effective and consistent practice.[3,4] The BSH recommends POCT delivery in secondary care within a structured governance model that meets these requirements, supporting reliable results, patient safety and sustainable service delivery.

INFECTED BLOOD INQUIRY

The Infected Blood Inquiry (IBI) examined how individuals were infected with HIV and hepatitis through blood and

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blood products, identifying serious systemic failures. Its final report calls for stronger transparency, accountability and safety systems to ensure such a tragedy cannot happen again. In line with the safety recommendations from the IBI, a culture of safety has been adhered to the implementation of this guideline.

Notes relating to this can be found in the guidelines development process document.

AUTHOR CONTRIBUTIONS

Jennifer Tam: Writing – review and editing. **Barbara De la Salle:** Writing – review and editing. **Patience Kapuya:** Writing – review and editing. **Deepa J. Arachchillage:** Writing – review and editing. **Wayne Thomas:** Writing – review and editing. **Tony Cambridge:** Writing – review and editing. **Christopher Reilly-Stitt:** Writing – review and editing. **Tracey Smith-Straney:** Writing – review and editing.

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The BSH paid the expenses incurred during the writing of this guidance. All authors have made a declaration of interests to the BSH and Task Force Chairs which may be viewed on request. The following members of the writing group: JT, BDS, PK, CR-S, TS-S, TC and WT have no conflicts of interest to declare. DJA received funding from Werfen, Stago and Sysmex to attend national and international meetings and received a speaker's fee from Werfen (California, USA).

DATA AVAILABILITY STATEMENT

Data sharing not applicable to this article as no datasets were generated or analysed during the current study.

REVIEW PROCESS

Members of the writing group will inform the writing group Chair if any new evidence becomes available that would alter the strength of the recommendations made in this document or render it obsolete. The document will be reviewed regularly by the relevant task force and the literature search will be re-run every 3 years to search systematically for any new evidence that may have become available. The document will be archived and removed from the BSH current guidelines website if it becomes obsolete. If new recommendations are made, an addendum will be published on the BSH guidelines website (https://b-s-h.org.uk/guidelines/).

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